

Including Practical issues presentations in SoFs

The burden of treatment and other practical issues related to how a treatment or test affects the patients' daily life are frequently missing in guidelines and in many tools for shared decision-making.

These practical issues have also been called the “work” of being a patient. An exploration of large database of patient's reports of burden of treatment and practical issues related to interventions, has identified 15 categories of practical issues ¹:

1. **Medication routine** (e.g., frequency and route)
2. **Tests and visits** (i.e., how many are required to implement an intervention or as follow-up)
3. **Procedure and device** (i.e., what procedures are required, are there any devices involved)
4. **Recovery and adaptation** (i.e., how will recovery typically look like, what additional effort is required, how to manage dependencies)
5. **Coordination of care** (i.e., how does an intervention practically fit with the rest of care)
6. **Adverse effect, drug interactions and antidotes** (i.e., are there specific adverse effects or interactions to manage or anticipate, what is involved to mitigate or manage them, are there existing antidotes to drug interventions)
7. **Physical well-being** (i.e., is there any physical distress that may come with managing practical issues relation to an intervention)
8. **Emotional well-being** (i.e., is there any emotional distress that may come with managing practical issues relation to an intervention)
9. **Pregnancy and nursing** (i.e., how may an intervention be adjusted to pregnancy and nursing)
10. **Costs and access** (i.e., who is paying, and what is required to access the recommendation, assuming it is available in one's context)
11. **Food and drinks** (e.g., are there specific directions regarding diet or other health habits)
12. **Exercise and activities** (e.g., how may an intervention impact one's ability to function in daily life or to exercise)
13. **Social life and relationships** (e.g., how may an intervention impact one's social life and interactions)
14. **Work and education** (e.g., how may an intervention impact one's work or education)
15. **Travel and driving** (e.g., how may an intervention impact the ability to travel or drive a vehicle)

Having data on the categories, which are relevant for an intervention, summarized in SoFs can serve support their translation into **tools for shared decision making with patients.** ^{2 3}

Examples of how this could be done are displayed in the next page:

A. Example of a SoF comparing SGLT-2 inhibitors vs Standard of Care, including numerical summary of the evidence of each outcome, followed by a display of relevant practical issue
(Source : MAGICapp: <https://app.magicapp.org/#/guideline/5288>)

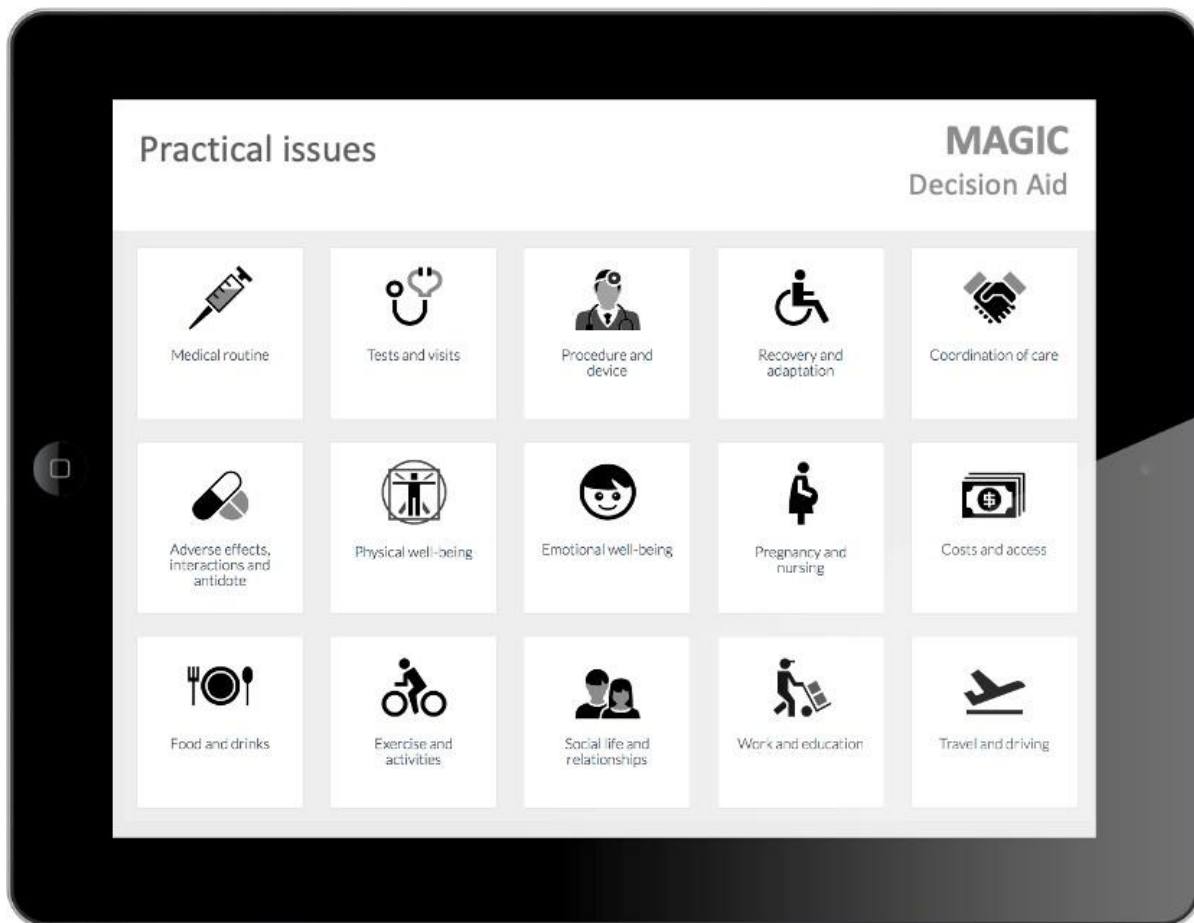
Sodium-Glucose Transport Protein 2 (SGLT2) inhibitors vs Standard care				
Adults with type 2 diabetes plus 3 more risk factors and no underlying CVD or CKD				
10 Outcomes 11 Practical issues				
Outcome Timeframe	Study results and measurements	Absolute effect estimates		Certainty of the evidence (Quality of evidence)
		Standard care	SGLT2 inhibitors	Plain language summary
All-cause mortality	Odds ratio 0.77 (CI 95% 0.71 — 0.83) Based on data from 282704 participants in 225 studies	70 per 1000	55 per 1000 Difference: 15 fewer per 1000 (CI 95% 19 fewer — 11 fewer)	High SGLT2 inhibitors reduce the risk of death compared with standard care.
Cardiovascular mortality	Odds ratio 0.84 (CI 95% 0.76 — 0.92) Based on data from 222944 participants in 128 studies	46 per 1000	39 per 1000 Difference: 7 fewer per 1000 (CI 95% 11 fewer — 4 fewer)	Moderate Due to serious imprecision. SGLT2 inhibitors probably reduce the risk of cardiovascular death compared with standard care.
Nonfatal myocardial infarction	Odds ratio 0.87 (CI 95% 0.79 — 0.97) Based on data from 222944 participants in 199 studies	58 per 1000	51 per 1000 Difference: 7 fewer per 1000 (CI 95% 12 fewer — 2 fewer)	Moderate Due to serious imprecision SGLT2 inhibitors probably reduce the risk of nonfatal myocardial infarction compared with standard care.
Sodium-Glucose Transport Protein 2 (SGLT2) inhibitors vs Standard care				
Adults with type 2 diabetes plus 3 more risk factors and no underlying CVD or CKD				
10 Outcomes 11 Practical issues				
Practical issues	SGLT2 inhibitors	Standard care	Both	
 Medication routine	Tablets swallowed once daily at the same time. Some need to be taken in the morning.	Most anti-diabetic medications are tablets except insulin, an injection.		
 Tests and visits	Needs to monitor blood pressure and kidney function.	Needs self-monitoring of blood glucose at home if the patients are receiving insulin, sulfonylureas or meglitinides.	Regular blood samples, and more frequently after starting a new medication. We only consider adding these drugs when the patient has HbA1C above 6.5.	
 Recovery and adaptation			Most people need to take diabetes medications for the rest of their lives. Should not be taken while a person is sick, especially if there is vomiting, diarrhea, or the person isn't eating and drinking very much.	
 Adverse effects, interactions and antidote	Mild to moderate urogenital infections can occur. Low blood pressure, dizziness, dehydration, etc., especially among people older than 65 years or when combined with diuretics. May not be as effective in patients with very poor kidney function. All patients should monitor for signs of diabetic ketoacidosis (even if the blood glucose is within the normal range) and seek hospital attention immediately if it occurs.	Adverse effects vary between specific agents, especially for insulin or insulin secretagogues can cause low blood sugar.	SGLT2 inhibitors or GLP-1 RAs do not increase the risk of hypoglycaemia when taken alone. But they do increase the risk of hypoglycaemia when used in combination with some medications such as insulin or insulin secretagogues.	

B. Example of how key practical issue practical issues can be displayed in an infographic
(Source : BMJ Rapid Recommendation : <https://www.bmj.com/content/373/bmj.n1091>)

Key practical issues		
Usual care	SGLT-2 inhibitors	GLP-1 receptor agonists
Most anti-diabetic medications are tablets except insulin, an injection	Should not be taken while a person is unwell, especially if there is vomiting, diarrhoea, or the person isn't eating and drinking much Tablets swallowed once daily. Some must be taken in the morning	Injection once or twice daily or once weekly Also available as tablets, but these are not yet widely available Formulas that combine GLP-1 receptor agonists and insulin are available

C. Example of practical issue displayed in a Decision Aid to support shared decision making in the clinical encounter

(Source: SHARE-IT Decision aids in MAGICapp: <https://magicvidence.org/decision-aids>)



References

- 1 Heen AF, Vandvik PO, Brandt L, *et al.* A framework for practical issues was developed to inform shared decision-making tools and clinical guidelines. *J Clin Epidemiol.* 2021;129:104–13. doi: 10.1016/j.jclinepi.2020.10.002
- 2 Agoritsas T, Heen AF, Brandt L, *et al.* Decision aids that really promote shared decision making: the pace quickens. *BMJ.* 2015;350:g7624. doi: 10.1136/bmj.g7624
- 3 Heen AF, Vandvik PO, Brandt L, *et al.* Decision aids linked to evidence summaries and clinical practice guidelines: results from user-testing in clinical encounters. *BMC Med Inform Decis Mak.* 2021;21:202. doi: 10.1186/s12911-021-01541-7